Cancer treatment within 62 days

Outturn for September 2026 · forecast issued with data through August 2026

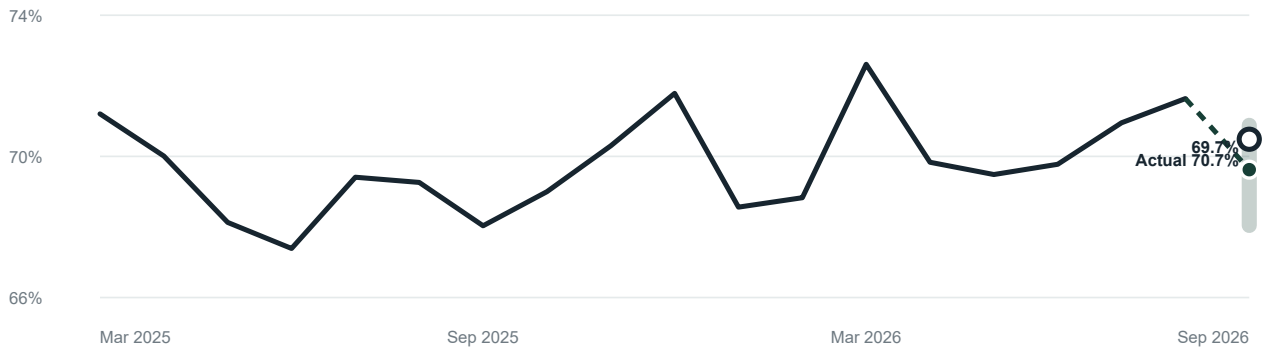
The published result was 70.7%. It worsened 1.2pp from the previous result and was 0.9pp better than outlook · inside the 80% range.

AT FORECAST · AUGUST 2026	FORECAST · SEPTEMBER 2026	ACTUAL
71.9%	**69.7%**	**70.7%**
Known when the forecast was made	80% expected range 68.0%–71.1%	Worsened 1.2pp from the previous result · 0.9pp better than outlook · inside the 80% range
		Off course · 80% by March 2027

Mar 2027 outlook at issue Projected **74.2%** Target **≥80%** Model-implied chance **4%**

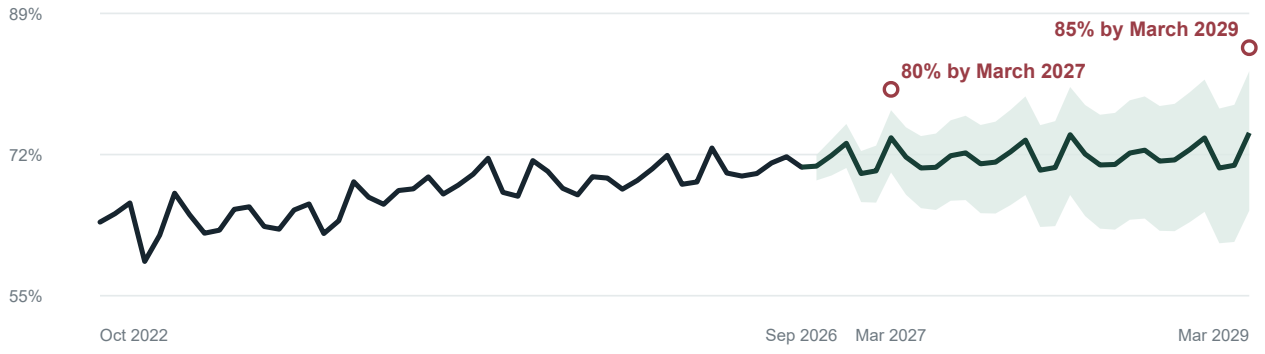
Recent performance and current release

The vertical band is the 80% expected range.



Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.



Benchmark: 85% constitutional standard. 6.0pp below milestone

Data: NHS England cancer waiting times. **Targets:** Medium Term Planning Framework interim target · Medium Term Planning Framework final target

Method: Backtest-weighted ensemble of current-level seasonal movement, damped annual drift and a damped recent seasonal trend; at least 2 components are required. The final fit uses 54 consecutive months. Accuracy is tested on 24 rolling one-month-ahead forecasts. The 80% range uses empirical historical error quantiles from 24 forecast errors. Weights use inverse rolling RMSE and are shrunk toward equal weights. The pre-release commentary compares the current-level seasonal component with the combined forecast. Provider distribution and trajectory watch are shown on page 2.

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Distribution and trajectory watch · September 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



Moderate = 1–<2×; marked = at least 2× the metric-specific materiality threshold.

Trajectory watch

Six actual months are compared with one forecast path fixed before that window. Both lines begin at the same observed value. A low performer can be improving, while a high performer can be deteriorating. Status: Actual performance is compared with the monthly path forecast six months earlier. The displayed figure is the latest actual minus the fixed forecast for that same month; the six-month window checks persistence.

114	31	16	67
providers assessed	sustained favourable	sustained adverse	without an active signal

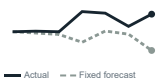
Largest sustained favourable moves

NORTH CUMBRIA INTEGRATED CARE NHS FOUNDATION TRUST
Latest 80.4% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



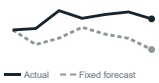
29.6pp
LATEST MONTH: ABOVE FORECAST

THE HILLINGDON HOSPITALS NHS FOUNDATION TRUST
Latest 83.4% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



23.6pp
LATEST MONTH: ABOVE FORECAST

SALISBURY NHS FOUNDATION TRUST
Latest 68.4% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



20.8pp
LATEST MONTH: ABOVE FORECAST

Largest sustained adverse moves

ISLE OF WIGHT NHS TRUST
Latest 66.4% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



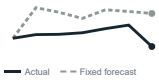
11.7pp
LATEST MONTH: BELOW FORECAST

BOLTON NHS FOUNDATION TRUST
Latest 67.5% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



10.6pp
LATEST MONTH: BELOW FORECAST

THE NEWCASTLE UPON TYNE HOSPITALS NHS FOUNDATION TRUST
Latest 61.5% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



9.6pp
LATEST MONTH: BELOW FORECAST

Trajectory-watch rule. Six actual months are compared with the corresponding path fixed before the six-month window. The displayed gap is the latest actual minus the fixed forecast for that same month. A signal requires a material latest-month gap and at least five of six months in that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.